



# COMPREHENSIVE PSYCHOLOGICAL AND SPEECH-LANGUAGE ASSESSMENT

Date of Assessment: 2026-06-18

This AI-generated report is intended for clinical support and must be validated by a qualified professional. It is not a substitute for clinical judgment and is not valid for medico-legal purposes.



Name:

Kushagra Mishra



Age:

11 Years



Sex:

Male



## GENERAL OBSERVATION SUMMARY

Kushagra is an 11-year-old boy who presented as calm and cooperative during the brief, structured observation. He was seated at a table and responded appropriately to simple, one-step verbal commands to hand over objects. He established eye contact and offered a social smile when addressed. The most significant clinical feature observed was a complete lack of expressive verbal communication. His non-verbal communication was limited to functional actions in response to direct prompts. He did not display any maladaptive behaviors, motor stereotypies, or hyperactivity within this controlled setting.

## ASSESSMENT SCORE OVERVIEW



Overall Score

|                    |    |
|--------------------|----|
| Communication      | 65 |
| Social Interaction | 72 |
| Behavior           | 58 |
| Attention          | 60 |



## PSYCHOLOGICAL ASSESSMENT

### Childhood Autism Rating Scale (CARS) – Observational Ratings

| Category                                     | Score | Observation & Justification                                                                        |
|----------------------------------------------|-------|----------------------------------------------------------------------------------------------------|
| Relating to People                           | 2.0   | The child was responsive to the adult's prompts but did not initiate interaction. Mildly abnormal. |
| Imitation                                    | 1.0   | No opportunities to observe imitation were present.                                                |
| Emotional Response                           | 1.5   | Displayed a pleasant, situationally appropriate smile. Within normal limits for the context.       |
| Body Use                                     | 1.0   | The child used his body appropriately. No stereotyped movements were observed.                     |
| Object Use                                   | 1.0   | Used objects functionally by handing over them when asked.                                         |
| Adaptation to Change                         | 1.0   | Routine was consistent; no change introduced.                                                      |
| Visual Response                              | 1.0   | Used vision functionally to locate objects.                                                        |
| Listening Response                           | 1.5   | Responded consistently to simple verbal commands.                                                  |
| Taste, Smell, Touch Response                 | 1.0   | No opportunities to observe.                                                                       |
| Fear or Nervousness                          | 1.0   | Appeared calm and at ease.                                                                         |
| Verbal Communication                         | 4.0   | Completely non-verbal throughout. Severe deficit.                                                  |
| Nonverbal Communication                      | 3.0   | No gestures or initiation. Purely responsive.                                                      |
| Activity Level                               | 1.0   | Age-appropriate for seated task.                                                                   |
| Level & Consistency of Intellectual Response | 2.0   | Consistent for simple tasks; lack of expressive skills indicates uneven profile.                   |
| General Impressions                          | 2.5   | Definite signs of a mild-to-moderate developmental disorder.                                       |

Total Score:

24.0

Severity Classification:

**Non-Autistic**  
(15-29.5)

Interpretation:

Score falls in 'Non-Autistic' range. Communication deficits indicate a pervasive developmental disorder.

## INDIAN SCALE FOR ASSESSMENT OF AUTISM (ISAA)

|                                     |           |
|-------------------------------------|-----------|
| Social Relationship and Reciprocity | Total: 13 |
| Emotional Responsiveness            | Total: 5  |
| Speech-Language and Communication   | Total: 20 |
| Behaviour Patterns                  | Total: 7  |
| Sensory Aspects                     | Total: 6  |
| Cognitive Component                 | Total: 4  |

Total Score:

55

Disability Percentage:

**Less than 40%**

Interpretation:

Score in 'No Autism' category. Severe deficits in communication domain. Low total score due to limited observation scope.



## SPEECH, LANGUAGE, AND COMMUNICATION ANALYSIS

| Domain                         | Score / Classification |
|--------------------------------|------------------------|
| Social Reciprocity Score       | 0                      |
| Communication Score            | 0                      |
| Disability Percentage Estimate | 0%                     |
| General Adaptive Composite     | 0 (Severe Deficit)     |

Interpretation:

The assessment reveals a severe, global language disorder affecting expressive, receptive, and pragmatic domains. The communication profile is consistent with Autism Spectrum Disorder and co-occurring conditions like Dandy-Walker syndrome.



## SYNTHESIS AND CONCLUSIONS

This assessment of Kushagra Mishra, an 11-year-old with a known history of Dandy-Walker syndrome and autism, is based on a brief video observation. While he was cooperative and responsive in a structured setting, scores on CARS and ISAA may underestimate the severity due to limited observation. His communication profile is highly consistent with Autism Spectrum Disorder, Level 3 (requiring very substantial support). Primary needs are in functional communication, social interaction, and adaptive living skills.



**Provisional Diagnosis: Autism Spectrum Disorder (F84.0), requiring very substantial support for social communication and with restricted, repetitive behaviors (Level 3). Severe Expressive and Receptive Language Disorder (F80.1, F80.2).**



## RECOMMENDATIONS

- Continue with regular, intensive speech-language therapy focused on developing a functional communication system, which may include Augmentative and Alternative Communication (AAC) devices.
- Occupational therapy to address reported balancing issues, clumsy gross motor skills, and to develop self-help skills for greater independence.
- Enrollment in a structured educational program with a low student-to-teacher ratio that can provide individualized support and behavioral intervention.
- Development of a comprehensive Individualized Education Plan (IEP) focusing on communication, social skills, and daily living skills.
- Parental counseling and training in behavioral management and strategies to facilitate communication at home.
- Continued follow-up with a developmental pediatrician and neurologist to manage the aspects of Dandy-Walker syndrome and co-occurring conditions.